

# Parvarish Clinical Knowledge Base: Adolescent Parenting Protocols

Structured Action-Response Pairs for High-Precision Vector Ingestion (Ages 9-19)

**System Note:** These protocols are specifically derived from the "Parvarish" modules of *The Art of Parenting* manual, tailored for pre-adolescents and teenagers in the Pakistani context.

## 1. Adolescent Communication & Behavioral Styles

[Module: BehaviorStyles | Stage: Adolescence | Category: DISC\_Model | Tags: communication, personality, conflict]

**Adolescent Trigger:** The adolescent exhibits distinct behavioral traits (Dominance, Influence, Steadiness, or Conscientiousness) requiring tailored parental communication.

### Evidence-Based Parent Protocol:

1. **For "D" (Dominance) Style:** Verbalize the desired result and provide them with space to deliver it independently. Refrain from micro-managing or giving long, detailed explanations.
2. **For "I" (Influence) Style:** Approach them in a personable, upbeat manner using humor. Limit excessive details and openly recognize their accomplishments.
3. **For "S" (Steadiness) Style:** Approach deliberately and warmly. Explain step-by-step processes and limit abrupt changes to plans.
4. **For "C" (Conscientious) Style:** Provide clear rules backed by facts. Avoid vague responses and avoid surprises.

[Module: Communication | Stage: Adolescence | Category: ActiveListening | Tags: validation, I\_statements]

**Adolescent Trigger:** The teenager expresses strong opinions, argues, or engages in behavior that breaks parental trust.

### Evidence-Based Parent Protocol:

1. **Use "I" Statements:** Communicate impact without harsh judgment. Say, "I feel sad because you broke my trust," rather than, "You are bad because you lied to me."
2. **Active Listening:** Stop other tasks, maintain eye contact, and let the teenager finish speaking before responding.
3. **Soften Reactions:** Avoid explosive anger, as adolescents will tune out or become defensive if parents appear hostile.

## 2. Psycho-Social Triage & Digital Safety

[Module: MentalHealth | Stage: Adolescence | Category: DepressionTriage | Tags: depression, warning\_signs, suicidal\_ideation]

**Adolescent Trigger:** The adolescent exhibits prolonged sadness, social withdrawal, loss of interest in activities, or a severe drop in academic performance.

### Evidence-Based Parent Protocol:

1. **Differentiate from Normal Sadness:** Normal sadness allows the child to remain hopeful and maintain connections. Depression involves excessive guilt, constant sadness, and a sense of hopelessness.
2. **Do Not Dismiss:** Never ignore suicidal thoughts, lethargy, or extreme isolation as "just a phase" of puberty.
3. **Seek Professional Help:** If signs of depression are persistent, consult mental health professionals immediately to mitigate self-harm risks.

[Module: DigitalSafety | Stage: Adolescence | Category: Cyberbullying | Tags: social\_media, bullying, intervention]

**Adolescent Trigger:** The teenager becomes secretive about digital life, avoids school, displays sudden mood drops after using the internet, or refuses homework.

### Evidence-Based Parent Protocol:

1. **Open Dialogue:** Sit down, offer support, and assure the child that the abuse is rarely their fault.
2. **Do NOT Confiscate Devices:** Never threaten to cut internet access or confiscate the phone as a reaction; this acts as a punishment and deters the child from reporting future bullying.
3. **Monitor Safely:** Keep the family computer in a busy area, use tracking software/filters, and review buddy lists to know who they communicate with online.

## 3. Cultural Dynamics, Gender & Family Conflict

[Module: GenderRoles | Stage: AllAges | Category: Stereotypes | Tags: gender\_equity, household\_chores, career]

**Family Trigger:** The family defaults to traditional gender roles (e.g., girls restricted to chores, boys discouraged from emotional expression or domestic tasks).

### Evidence-Based Parent Protocol:

1. **Break Stereotypes:** Provide equal opportunities. Fathers should model shared household responsibilities (e.g., cleaning, cooking) to dismantle restrictive norms.
2. **Career Autonomy:** Ask "What do you love to do?" rather than dictating careers based on gender or societal prestige.
3. **Educate on Consent:** Teach children the importance of personal boundaries and respecting the consent of others from an early age.

[Module: FamilyCrisis | Stage: PreTeen\_Teen | Category: Separation\_Divorce | Tags: loyalty\_conflict, co\_parenting]

**Family Trigger: Parents are undergoing separation or divorce; pre-teens (8-12) feel guilty, or teenagers (13-16) withdraw socially.**

**Evidence-Based Parent Protocol:**

1. **Alleviate Guilt:** Explicitly reassure 8-12 year olds that they are not the cause of the separation and that both parents still love them.
2. **Maintain Respect:** Demonstrate mutual respect towards the ex-partner during family gatherings (e.g., Eid, weddings) to set a strong example and prevent loyalty conflicts.
3. **Preserve Routines:** Keep the adolescent's school, sports, and social routines entirely intact to provide stability.

## 4. Bereavement & Chronic Illness Management

[Module: GriefManagement | Stage: AllAges | Category: Death\_Bereavement | Tags: DABDA, coping, communication]

**Child Trigger: A close family member dies; the child experiences shock, anger, guilt, or depression.**

**Evidence-Based Parent Protocol:**

1. **Use Clear Language:** Use concrete words like "died" instead of confusing metaphors.
2. **Validate the Grieving Process:** Acknowledge that anger, guilt, and depression are natural stages of grief (DABDA). Do not force immediate positivity.
3. **Acknowledge Secondary Losses:** Validate the pain of missing the deceased on future milestones (birthdays, graduations).
4. **Consistency:** Maintain daily routines and reliable caregiving to provide comfort.

[Module: MedicalCare | Stage: AllAges | Category: ChronicIllness | Tags: hospitalization, autonomy, peer\_support]

**Child Trigger: The child is diagnosed with a chronic illness (e.g., diabetes, asthma) requiring intense management and potential hospitalization.**

**Evidence-Based Parent Protocol:**

1. **Avoid Overprotection:** Resist being overly permissive or overly strict. Provide the child with flexible choices regarding their routine to promote independence.
2. **Honest Disclosure:** Discuss the illness, treatments, and side effects honestly at an age-appropriate level to prevent the child from imagining worst-case scenarios.
3. **School Coordination:** Develop a "whole school" approach by communicating specific medical and emotional needs to teachers and counselors.
4. **Peer Preparation:** Help the child practice scripts for what to say to classmates who ask questions or stare.